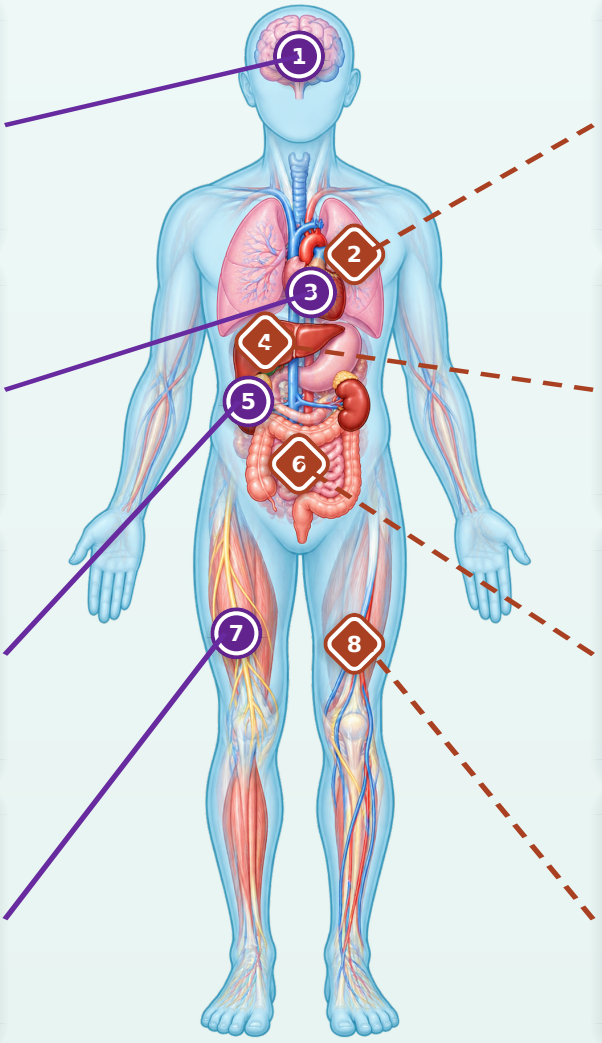
Nivolumab, ipilimumab, pemetrexed and cisplatin

The first cycle lasts 42 days: all four medicines on Day 1, then nivolumab, pemetrexed and cisplatin on Day 22. Chemotherapy ends after those two visits. From Cycle 2, nivolumab continues on Days 1 and 22 and ipilimumab on Day 1 of each 42-day cycle. Immune treatment continues while appropriate until the cancer progresses, side effects require stopping, or up to 24 months without progression. Plans can change after review.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Hormones / brain

Nivolumab / ipilimumab

Unusual weakness, headache, dizziness, thirst or passing much more urine.

2 Lungs

Nivolumab / ipilimumab

New cough or breathlessness needs prompt assessment; infection or cancer can cause similar symptoms.

3 Heart / muscles

Nivolumab / ipilimumab

Chest pain, racing heartbeat, fainting or new muscle weakness needs urgent assessment.

4 Liver

Nivolumab / ipilimumab

Yellow eyes/skin, dark urine or abnormal tests can signal inflammation.

5 Kidneys

Pemetrexed / Cisplatin / immune medicines

Kidney tests matter; report less urine, swelling or difficulty drinking.

6 Bowel / mouth

Pemetrexed / nivolumab / ipilimumab

Report new diarrhoea, pain, blood/mucus or mouth ulcers; do not assume ordinary chemotherapy upset.

7 Skin / nerves / hearing

Immune medicines / Cisplatin

Report spreading rash, blistering, weakness, numbness or ringing/ hearing changes.

8 Blood / infection

Pemetrexed / Cisplatin

Fever, shivers, sudden illness or unusual bleeding: contact oncology immediately.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Follow your oncology team’s emergency instructions for fever, shivering or sudden illness, including with normal temperature. Report new breathlessness/cough, diarrhoea or abdominal pain, reduced urine, severe rash, unusual weakness, headache or marked thirst promptly. Severe chest pain, collapse, confusion or major breathing difficulty needs emergency care.

Your treatment, at home.

Important medicine-specific effects

Nivolumab

- Immune inflammation in multiple organs
- Infusion reactions
- Delayed effects after treatment stops

Ipilimumab

- Immune bowel, liver, skin and hormone inflammation
- Other serious organ inflammation
- Infusion reactions

Pemetrexed

- Low blood counts and infection
- Mouth soreness, rash and fatigue
- Kidney toxicity

Cisplatin

- Low blood counts and infection
- Nausea and vomiting
- Kidney, hearing and nerve effects
- Allergic reactions

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

Cycle 1 Days 1 and 22 · chemotherapy-containing visits

Use generic high-risk NCIS row including olanzapine. Cisplatin requires pre/post hydration and electrolyte review; follow individual/local fluid policy. Folic acid starts before pemetrexed, continues through both chemotherapy visits and for 21 days after the last pemetrexed dose. B12 is given in the preceding week. The source repeats B12 every three cycles if pemetrexed continues; this planned course has only two pemetrexed administrations, so do not automatically continue B12 in immune-only cycles. Your team reconciles the steroid used to prevent pemetrexed rash with the anti-sickness steroid into one clear prescription. Do not add the two schedules together or use leftover steroids yourself. Nivolumab and ipilimumab can inflame healthy organs. Report new symptoms promptly, including after treatment stops; follow-up vigilance is required for at least five months after the last dose.

Cycle 2 onwards Day 1 · nivolumab + ipilimumab every 42 days

Ipilimumab is low risk and nivolumab minimal; this combined visit is low risk. No further pemetrexed or platinum: stop/review induction antiemetics and rash steroid; complete the folate tail only. Continue immune symptom education, bloods each treatment and glucose/TFT each 42-day cycle.

Cycle 2 onwards Day 22 · nivolumab alone, between ipilimumab visits

No routine prophylactic antiemetic course for nivolumab-only minimal-risk treatment; prescribe symptom-directed rescue under local policy if needed. Ipilimumab is due at the next 42-day cycle Day 1, not every nivolumab visit. Immune toxicity can still occur between ipilimumab visits; the joint hold rule remains relevant.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00714/

Sources and review status are available online. Follow your own prescription and hospital instructions.